

Tamil Nadu Medical Council (Professional Conduct, Etiquette and Ethics) Regulations, 2003

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State of Tamilnadu- Act

Tamil Nadu Medical Council (Professional Conduct, Etiquette and Ethics) Regulations, 2003

TAMILNADU

India

Tamil Nadu Medical Council (Professional Conduct, Etiquette and Ethics) Regulations, 2003

Rule

TAMIL-NADU-MEDICAL-COUNCIL-PROFESSIONAL-CONDUCT-ETIQUETTE-AND-ETHICS-REGULATIONS-2003 of 2003

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Tamil Nadu Medical Council (Professional Conduct, Etiquette and Ethics) Regulations, 2003

Chapter I

A. Declaration

1.

Code of Medical Ethics.

Each applicant at the time of making an application for registration under the provisions of the Tamil Nadu Medical Registration Act, 1914 (Tamil Nadu Act IV of 1914), shall be provided a copy of the Declaration and shall submit a duly

signed Declaration as provided in Appendix 1. The applicant shall also certify that he/she had read and agreed to abide by the same.

B. Duties and Responsibilities of the Physician in General

Character of the Physician

1.1

(Doctors with qualification of MBBS or MBBS with post graduate degree/diploma or with the equivalent qualification in any medical discipline).

1.1.1

. A physician shall uphold the dignity and honour of his profession.

1.1.2

. The prime object of the medical profession is to render service to humanity; reward or financial gain is a subordinate consideration. Whosoever chooses his profession, assumes the obligation to conduct himself in accordance with its deals.

A physician should be an upright man, instructed in the art of healings. He shall keep himself pure in character and be diligent in caring for the sick; he should be modest, sober, patient, prompt in discharging his duty without anxiety, conducting himself with propriety in the profession and in all the actions of his life.

1.1.3

. No person other than a doctor having qualification recognized by Medical Council of India and registered with Tamil Nadu Medical Council is allowed to practise modern system of Medicine or Surgery. A person obtaining qualification in any other system of Medicine is not allowed to practise Modern System in any form.

Maintaining Good Medical Practice

1.2.1

. The principal objective of the medical profession is to render service to humanity with full respect for the dignity of profession and man. Physicians should merit the confidence of patients entrusted in their care, rendering to each a full measure of service and devotion.

Physicians should try continuously to improve medical knowledge and skills and should make available to their patients and colleagues the benefits of their professional attainments.

The physicians should practise methods of healing founded on scientific basis and should not associate professionally with anyone who violates this principle. The honoured ideals of the medical profession imply that the responsibilities of the physician extend not only to individuals, but also to society.

Membership in Medical Society

1.2.2

. For the advancement of his profession, a physician should affiliate with associations and societies of allopathic medical professions and invoice actively in the functioning of such bodies.

1.2.3

. A physician should participate in professional meetings as part of Continuing Medical Education Programmes, for at least 30 hours for every five years, organized by reputed professional academic bodies or any other authorized organisations. The compliance of this requirement shall be informed regularly to the Tamil Nadu Medical Council.

Maintenance of Medical Records

1.3.1

. Every physician shall maintain the medical records pertaining to his/her indoor patients for a period of 3 years from the date of commencement of the treatment in a standard proforma laid down by the Tamil Nadu Medical Council and attached as Appendix 3.

1.3.2

. If any request is made for medical records either by the patients/ authorised attendant or legal authorities involved, the same may be duly acknowledged and documents shall be issued within the period of 72 hours.

1.3.3

. A Registered medical practitioner shall maintain a Register of Medical Certificates giving full details of certificates issued. When issuing a medical certificate, he/she always enter the identification marks of the patient and keep a copy of the certificate. He/she shall not omit to record the signature and/or thumb mark, address and at least one identification mark of the patient on the medical certificates or report. The medical certificate shall be prepared as in Appendix 2.

1.3.4

. Efforts shall be made to computerize medical records for quick retrieval.

Display of Registration Number

1.4.1

. Every physician shall display the registration number accorded to him by the Tamil Nadu Medical Council in his clinic and in all his prescriptions, certificates, money receipts given to his patients.

1.4.2

. Physician shall display as suffix to their names only recognised medical degrees or such certificates/diplomas and membership/honours, which confer professional knowledge, or recognise any exemplary qualification/ achievements.

Use of Generic Names of Drugs

1.5

Every physician should, as far as possible, prescribe drugs with generic names and he/she shall ensure that there is a rational prescription and use of drugs.

Highest Quality Assurance In Patient Care

1.6

Every physician should aid in safeguarding the profession against admission to it of those who are deficient in moral character or education. Physician shall not employ in connection with his professional practice any attendant who is neither registered nor enlisted under the Medical lists in force and shall not permit such persons to attend, treat or perform operations upon patients wherever professional discretion or skill is required.

Exposure of Unethical Conduct

1.7

A physician should expose, to the appropriate authority without fear or favour, incompetent or corrupt, unprofessional, dishonest or unethical conduct on the part of members of the professions.

Payment of Professional Services

1.8

The physician engaged in the practice of medicine shall give priority to the interests of patients. The personal financial interests of a physician should not conflict with the medical interests of patients. A physician should announce his fees before rendering service and not after the operation or treatment is under way.

Remuneration received for such services should be in the form and amount specifically announced to the patient at the time the service is rendered. It is unethical to enter into a contract of "no cure no payment". Physician rendering services on behalf of the State shall refrain from anticipating or accepting any consideration.

Evasion of Legal Restrictions

1.9

The physician will observe the laws of the country in regulating the practice of medicine and will also not assist others to evade such laws. He should be cooperative in observance and enforcement of sanitary laws and regulations in the interest of public health.

Legislation's and Court decisions may also influence medical ethics.

Hence, Medical Practitioners must be familiar with the various Acts, Rules and Regulations that are in force and which have been enacted by the Central or State Government, Statutory bodies, Tamil Nadu Medical Council and the Indian Medical Council.

Unless he is familiar with all the relevant laws and regulations, he may find himself inadvertently contravening these and courting trouble.

A physician should observe the provisions of the State Acts like Drugs and Cosmetics Act, 1940; Pharmacy Act, 1948; Narcotic Drugs and Psychotropic Substances Act, 1985; Medical Termination of Pregnancy Act, 1971; Transplantation of Human Organs Act, 1994; Mental Health Act, 1987; Environmental Protection Act 1986; Pre-natal Sex Determination Test Act, 1994; Drugs and Magic Remedies (Objectionable Advertisement) Act, 1954; Persons with Disabilities (Equal Opportunities and Full Participation) Act, 1995 and the Bio-Medical Waste (Management and Handling) Rules, 1998 and such other Acts, Rules, Regulations made by the Central/State Governments or local Administrative Bodies or any other relevant Act relating to the protection and promotion of public health.

Chapter 2

Obligations to the Sick

2.

Duties of Physicians to their Patients.

2.1.1

. Though a physician is not bound to treat each and every person asking his services, he should not only be ever ready to respond to the calls of the sick and the injured, but should be mindful of the high character of his mission and the responsibility be discharges in the course of his professional duties. In his treatment, he should never forget that the health and the lives of those entrusted to his care depend on his skill and attention.

A physician shall, while caring for a patient, regard responsibility to the patient as paramount.

A physician should endeavour to add to the comfort to the sick by making his visits at the hour indicated to the patients.

A physician advising a patient to seek service of another physician is acceptable, however, in case of emergency a physician must treat the patient.

No physician shall arbitrarily refuse treatment to a patient. However, for good reason, when a patient is suffering from an ailment is not within the range of experience of the treating physician, the physician, may refuse treatment and refer the patient to another physician.

2.1.2

. Medical practitioner having any incapacity detrimental to the patient or which can affect his performance vis-a-vis the patient is not permitted to practise his profession.

Patience, Delicacy and Secrecy

2.2

Patience and delicacy should characterize the physician. Confidence concerning individual or domestic life entrusted by patients to a physician and defects in the disposition or character of patients observed during medical attendance should never be revealed unless their revelation is required by the laws of the State. Sometimes, however, a physician must determine whether his duty to society requires him to employ knowledge, obtained through confidence as a physician, to protect a healthy person against a communicable disease to which he is about to be exposed. In such instance, the physician should act as he would wish another to act toward one of his own family in like circumstances.

Prognosis

2.3

The physician should neither exaggerate nor minimize the gravity of a patients condition. He should ensure himself that the patient, his relatives or his responsible friends have such knowledge of the patients condition as well serve the best interests of the patient and the family.

The Patient Must not be Neglected

2.4

A physician is free to choose whom he will serve. He should, however, respond to any request for his assistance in an emergency. Once having undertaking a case, the physician should not neglect the patient, nor should he withdraw from the case without giving adequate notice to the patient and his family. Provisionally or fully registered medical practitioners shall not willfully commit an act of negligence that may deprive his patient or patients from necessary medical care.

Engagement for an Obstetric Case

2.5

When a physician who has been engaged to attend an obstetric case is absent and another is sent for and delivery accomplished, the acting physician is entitled to his professional fees, but should secure the patients consent to resign on the arrival of the physician engaged.

Chapter 3

Duties of Physician in Consultation Unnecessary Consultations

Should be Avoided

3.1.1

. However in case of serious illness and in doubtful or difficult conditions, the physician should request consultation, but under any circumstances, such consultation should be justifiable and in the interest of the patients only and not for any other consideration.

3.1.2

. Consulting Pathologists/Radiologists or asking for any other diagnostic Lab investigation should be done judiciously

and not in a routine manner.

Consultation for Patient's Benefit

3.

2. In every consultation, the benefit to the patient is of foremost importance. All physicians engaged in the case should be frank with the patient and his attendants.

Punctuality in Consultation

3.3

Utmost punctuality should be observed by a physician in making themselves available for consultations.

Statement to Patient After Consultation

3.4.1

. All statements to the patient or his representative should take place in the presence of the consulting physicians, except as otherwise agreed. The disclosure of the opinion to the patient or his relatives or friends shall rest with the medical attendant.

3.4.2

. Difference of opinion should not be divulged unnecessarily, but when there is irreconcilable difference of opinion, the circumstances should be frankly and impartially explained to the patient or his relatives or friends. It would be opened to them to seek further advice as they so desire.

Treatment after Consultation

3.5

No decision should restrain the attending physician from making such subsequent variations in the treatment, if any, unexpected change occurs, but at the next consultation, reasons for the variations should be discussed/ explained. The same privilege, with its obligations, belongs to the consultant when sent for in an emergency during the absence of attending physician. The attending physician may prescribe medicine at any time for the patient, whereas the consultant may prescribe only in case of emergency or as an expert when called for.

Patients Referred to Specialists

3.6

When a patient is referred to a specialist by the attending physician, a case summary of the patient should be given to the specialist, who should communicate his opinion in writing to the attending physician.

Fees and Other Charges

3.7.1

. A physician shall inform his fees and other charges before rendering service and not after the operation or treatment underway. Prescription should also make clear if the physician himself dispensed any medicine.

3.7.2

. A physician shall write his name and designation in full along with registration particulars in his prescription letter-head.

Chapter 4

Responsibilities of Physicians to Each Other

Dependence of Physicians on Each Other

4.

1. A physician should consider it as a pleasure and privilege to render gratuitous service to all physicians and their immediate family dependants.

Conduct in Consultation

4.2

In consultations, no insincerity, rivalry or envy should be indulged in. All due respect should be observed towards the physician in charge of the case and no statement or remark be made, which impair the confidence reposed in him. For this purpose, no discussion should be carried on in the presence of the patient or his representatives.

The physician should refrain from making comments which may needlessly damage the reputation of a colleague or cause a patient anxiety.

Consultant not to take Charge of the Case

4.3

When a physician has been called for consultation, the consultant should normally not take charge of the case, especially on the solicitation of the patient or friends. The Consultant shall not criticize the referring physician. He/she

shall discuss the diagnosis treatment plan with the referring physician.

Appointment of Substitute

4.4

Whenever a physician requests another physician to attend his patients during his temporary absence from his practice, professional courtesy requires the acceptance of such appointment only when he has fine capacity to discharge the additional responsibility along with his/her other duties. The physician acting under such an appointment should give the utmost consideration to the interests and reputation of the absent physician and all such patients should be restored to the care of the latter upon his/her return.

Visiting Another Physician's Case

4.5

When it becomes the duty of a physician occupying an official position to see and report upon an illness or injury, he should communicate to the physician in attendance so as to give him an option of being present. The medical officer/physician occupying an office position should avoid remarks upon the diagnosis or the treatment that has been adopted.

Chapter 5

Duties of Physician to the Public and to the Paramedical

Profession Physicians as Citizens

5.

1. Physicians as good citizens, possessed of special training should disseminate advice on public health issues. They should play their part in enforcing the laws of the community and in sustaining the institutions that advance the interests of humanity. They should particularly co-operate with the authorities in the administration of sanitary/public health laws and regulations.

Public and Community Health

5.2

Physicians, especially those engaged in public health work, should enlighten the public concerning quarantine regulations and measures for the prevention of epidemic and communicable diseases.

At all times the physicians should notify the constituted public authorities of every case of communicable disease under his care, in accordance with the laws, rules and regulations of the health authorities. When an epidemic occurs, a physician should not abandon his duty for fear of contracting the disease himself.

Pharmacists/ Nurses

5.3

Physicians should recognize and promote the practice of different services such as, pharmacy and nursing as professions and should seek their cooperation wherever required.

Chapter 6

Unethical Acts

A physician shall not aid or abet or commit any of the following acts which shall be construed as unethical.

Advertising

6.

1. Solicitation of patients directly or indirectly, by a physicians, by a group of physicians or by institutions or organisations is unethical.

A physician shall not make use of him/her (or his/her name) as subject of any form or manner of advertising or publicity through any mode either alone or in conjunction with others which is of such a character as to invite attention to him or to his professional position, skill, qualification, achievements, attainments, specialities, appointments, associations, affiliations or honours and/or of such character as would ordinarily result in his self advertisement.

A physician shall not give to any person, whether for compensation or otherwise, any approval, recommendation, endorsement, certificate, report or statement with respect of any drug, medicine, nostrum, remedy, surgical, or therapeutic article, apparatus or appliance to any commercial product or article with respect of any property, quality or use thereof or any test, demonstration or trial thereof, for use in connection with his name, signature, or photograph in any form or manner of advertising through any mode nor shall be boast of cases, operations, cures or remedies or permit the publication of report thereof through any mode.

A medical practitioner is, however, permitted to make a formal announcement (without self photograph) in press

regarding the following :-

- (1)
On starting practice.
- (2)
On change of type of practice.
- (3)
On changing address.
- (4)
On temporary absence from duty.
- (5)
On resumption of another practice.
- (6)
On succeeding to another practice.
- (7)
Public declaration of charges.

6.1.2

. Printing of self photograph, or any such material of publicity in the letter head or on sign board of the consulting room or any such clinical establishment shall be regarded as acts of self advertisement and unethical conduct on the part of the physician. However, printing of sketches, diagrams, picture of system shall not be treated as unethical.

Patent and Copyrights

6.2

A physician may patent surgical instruments, appliances and medicine or copyright applications, methods and procedures. However, it shall be unethical if the benefits of such patents or copyrights are not made available situations where the interest of large population is involved.

Running an Open Shop (Dispensing of Drugs and Appliances by Physicians)

6.3

A physician should not run an open shop for sale of medicine for dispensing prescriptions prescribed by doctors other than himself or for sale of medical or surgical appliances.

It is not unethical for a physician to prescribe or supply drugs, remedies or appliances as long as there is no exploitation of the patient. Drugs prescribed by a physician or brought from the market for a patient should explicitly state the proprietary formula as well as generic name of the drug.

Rebates and Commission

6.4.1

. A physician shall not give, solicit, or receive nor shall be offer to give solicit or receive, any gift, gratuity, commission or bonus in consideration of or return for the referring, recommending or procuring of any patient for medical, surgical or other treatment.

A physician shall not directly or indirectly, participate in or be a party to act of division, transference, assignment, subordination, rebating, splitting or refunding of any fee for medical, surgical or other treatment.

6.4.2

. Provisions of para 6.4.1 shall apply with equal force to the referring, recommending or procuring by a physician or any person, specimen or material for diagnostic purposes or other study/work. Nothing in this section, however, shall prohibit payment of salaries by a qualified physician to other duly qualified person rendering medical care under his supervision.

Secret Remedies

6.5

The prescribing or dispensing by a physician of secret remedial agents of which he does not know the composition, or the manufacture or promotion of their use is unethical and as such prohibited. All the drugs prescribed by a physician should always carry a proprietary formula and clear name.

Human Rights

6.6

The physician shall not aid or abet torture nor shall he be a party to either infliction of mental or physical trauma or concealment or torture inflicted by some other person or agency in clear violation of basic human rights.

Euthanasia

6.7

Practising euthanasia shall constitute unethical conduct. However, on specific occasion, the question of withdrawing supporting devices to sustain cardio-pulmonary function even after brain death, shall be decided only by a team of doctors and not merely by the treating physician alone. A team of doctors shall declare withdrawal of support system. Such team shall consist of the doctor or the patient, Chief Medical Officer/Medical Officer in-charge of the hospital and a doctor nominated by the in-charge of the hospital staff or in accordance with the provisions of the Transplantation of Human Organ Act, 1994.

Chapter 7

Misconduct

The following acts of commission or omission on the part of a physician shall constitute professional misconduct rendering him/her liable for disciplinary action.

Violation of the Regulations

7.

1. If he/she commits any violation of these Regulations.

7.2

If he/she does not maintain the medical records of his/her indoor patients for a period of three years as per regulation 1.3 and refuses to provide the same within 72 hours when the patient or his/her authorised representative makes a request for it as per the regulation 1.3.2.

7.3

If he/she does not display the registration number accorded to him/ her by the Tamil Nadu Medical Council in his clinic, prescriptions and certificates, etc., issued by him or violates the provisions of regulation 1.4.2 (display of recognised medical degrees).

Adultery or Improper Conduct

7.4

Abuse of professional position by committing adultery or improper conduct with a patient or by maintaining an improper association with a patient will render a physician liable for disciplinary action as provided under the Tamil Nadu Medical Registration Act, 1914 or Indian Medical Council Act, 1956.

Conviction by Court of Law

7.5

Conviction by a Court of Law for offences involving moral turpitude/ criminal acts.

Sex Determination Tests

7.6

On no account, sex determination test shall be undertaken with the intent to terminate the life of a female foetus developing in her mother's womb, unless there are other absolute indications for termination of pregnancy as specified in the Medical Termination of Pregnancy Act, 1971. Any act of termination of pregnancy of normal female foetus amounting to female foeticide shall be regarded as professional misconduct on the part of the physician leading to penal measure, besides rendering him liable to criminal proceedings as per the provisions of this Act.

Signing Professional Certificates, Reports and Other Documents

7.7

Registered medical practitioners are, in certain cases, bound by law to give, or may, from time to time, be called upon or requested to give certificates, notification, reports and other documents of similar character signed by them in their professional capacity for subsequent use in the Courts or for administrative purposes, etc. Such documents, among others, include the ones given at Appendix 4.

Any registered practitioner who is shown to have signed or given his name and authority any such certificate, notification, report or document of a similar character which is untrue, misleading or improper is liable to have his name deleted from the Register.

7.8

A registered medical practitioner shall not contravene the provisions of the Drugs and Cosmetics Act and Regulations made thereunder. Accordingly -

(a)

prescribing steroids/psychotropic drugs when there is no absolute medical indication.

(b)

selling Schedule "H" & "E" drugs and poisons to the public except to his patient;

in contravention of the above provisions shall constitute gross professional misconduct on the part of the physician.

7.9

Performing or enabling unqualified person to perform an abortion or any illegal operation for which there is no medical, surgical or psychological indication.

7.10

A registered medical practitioner shall not issue certificates of efficiency in modern medicine to unqualified or non-medical person.

7.11

A physician should not contribute to the lay press articles and give interviews regarding diseases and treatments which may have the effect of advertising himself or soliciting practices, but is open to write to the lay press under his own name on matters of public health, hygienic living or to deliver public lectures, give talks on the radio/TV/internet chat for the same purpose and send announcement of the same to lay press.

7.12

An institution run by a physician for a particular purpose such as a maternity home, nursing home, private hospital, rehabilitation centre or any type of training institution, etc., may be advertised in the lay press, but such advertisements should not contain anything more than the name of the institution, type of patients admitted, type of training and other facilities offered and the fees.

7.13

It is improper for a physician to use an unusually large sign board and write on it anything other than his name, qualification obtained from a University or a statutory body, titles and name of his speciality, registration number including the name of the State Medical Council under which registered. The same should be the contents of his prescription papers.

It is improper to affix a sign-board on a chemist's shop or in places where he does not reside or work.

7.14

The registered medical practitioner shall not disclose the secrets of a patient that have been learnt in the exercise of his/her profession except -

(i)

in a Court of law under orders of the Presiding Judge;

(ii)

in circumstances where there is a serious and identified risk to a specific person and/or community; and

(iii)

notifiable diseases.

In case of communicable/notifiable diseases, concerned public health authorities should be informed immediately.

Should Not Refuse To Give Professional Service On Religious Grounds

7.15

The registered medical practitioner shall not refuse on religious grounds alone to give assistance in or conduct of sterility, birth control, circumcision and medical termination of pregnancy when there is medical indication, unless the medical practitioner feels himself/herself incompetent to do so.

7.16

Before performing an operation the physician should obtain in writing the consent from the husband or wife, parent or guardian in the case of minor, or the patient himself, as the case may be. In an operation which may result in sterility, the consent of both husband and wife is needed.

7.17

A registered medical practitioner shall not publish photographs or case reports of his/her patients without their permission, in any medical or other journal in a manner by which their identity could be made out. If the identity is not to be disclosed, the consent is not needed.

7.18

In the case of running of a nursing home by a physician and employing assistants to help him/her, the ultimate

responsibility rests on the physician.

7.19

A physician shall not use touts or agents for procuring patients.

7.20

A physician shall not claim to be specialist unless he has special qualification in that branch.

7.21

No act of invitro fertilization or artificial insemination shall be undertaken without the informed consent of the female patient and her spouse as well as the donor. Such consent shall be obtained in writing only after the patient is provided, at her own level of comprehension, with sufficient information about the purpose, methods, risks, inconveniences, disappointments of the procedure and possible risks and hazards.

7.22

Research. - Clinical drug trials or other research involving patients or volunteers as per the guidelines of ICMR can be undertaken, provided ethical considerations are borne in mind. Violation of existing ICMR guidelines in this regard shall constitute misconduct. Consent taken from the patient for trial of drug of therapy which is not as per the guidelines shall also be construed as misconduct.

Chapter 8

Punishment and Disciplinary Action

8.

1. It must be clearly understood that the instances of offences and of professional misconduct which are given above do not constitute and are not intended to constitute a complete list of the infamous acts which calls for disciplinary action, and that by issuing this notice the Tamil Nadu Medical Council/Medical Council of India are in no way precluded from considering and dealing with any other form of professional misconduct on the part of a registered medical practitioner. Circumstances may and do arise, from time to time, in relation to which there may occur questions of professional misconduct which do not come within any of these categories. Every care should be taken that the code is not violated in letter or spirit. In such instances as in all others, the Tamil Nadu Medical Council/Medical Council of India have to consider and decide upon the facts brought before the Council(s).

8.2

It is made clear that any complaint with regard to professional misconduct can be brought before the Tamil Nadu Medical Council for disciplinary action.

Upon receipt of any complaint of professional misconduct, the Tamil Nadu Medical Council would hold an enquiry and give opportunity to the registered medical practitioner to be heard in person or by pleader.

If the medical practitioner is found to be guilty of committing professional misconduct, the Tamil Nadu Medical Council may award such punishment as deemed necessary or may direct the removal altogether or for a specified period, from the register of the name of the delinquent registered practitioner. Deletion from the Register shall be widely published in local press as well as in the publications of different Medical Associations/Societies/Bodies.

8.3

In case the punishment of removal from the register is for a limited period, the Tamil Nadu Medical Council may also direct that the name so removed shall be restored in the register after the expiry of the period for which the name was ordered to be removed.

8.4

Decision on complaint against delinquent physician shall be taken within a time limit of 6 months.

8.5

During the pendency of the complaint, the Tamil Nadu Medical Council may restrain the physician from performing the procedure or practice which is under scrutiny.

8.6

Professional incompetence shall be judged by peer group as per guidelines prescribed by Tamil Nadu Medical Council/Medical Council of India.

Appendix-1

A. Declaration

At the time of registration, each applicant shall be given a copy of the following declaration by the Registrar concerned and the applicant shall read and agree to abide by the same :-

(1)

I solemnly pledge myself to consecrate my life to service of humanity.

(2)

Even under threat, I will not use my medical knowledge contrary to the laws of humanity.

(3)

I will maintain the utmost respect for human life from the time of conception.

(4)

I will not permit considerations of religions, nationality, race, party politics or social standing to intervene between my duty and my patient.

(5)

I will practise my profession with conscience and dignity.

(6)

The health of my patient will be my first consideration.

(7)

I will respect the secrets which are confided in me.

(8)

I will give to my teacher the respect and gratitude which is their due.

(9)

I will maintain by all means in my power, the honour and noble traditions of medical profession.

(10)

I will treat my colleagues with, all respect and dignity.

(11)

I shall abide by the code of medical ethics as enunciated in the Tamil Nadu Council (Professional Conduct, Etiquette and Ethics) Regulations, 2003.

I make these promises solemnly, freely and upon my honour.

Date.....

Signature.....

Place.....

Name.....

Address.....

Appendix-2

Form of certificate recommended for leave or extension or communication of leave and for fitness

Signature of patient or thumb impression

To be filled in by the applicant in the presence of the Government Medical Attendant, or Medical Practitioner.

Identification Marks:

1.

.....

2.

.....

I, Dr.....after careful examination of the case, certify hereby that..... whose signature is given above is suffering from..... and I consider that a period of absence from duty of.....with effect from.....is absolutely necessary for the restoration of his/her health.

I, Dr.....after careful examination of the case, certify hereby the.....

On restoration of health is now fit to join service.

Place.....Date.....

Signature of Medical Practitioner.Registration No.....

(Tamil Nadu Medical Council/Medical Council of India)

Note. - The nature and probable duration of the illness should also be specified. This certificate must be accompanied by a brief resume of the case giving the nature of the illness, its symptoms, causes and duration.

Appendix-3

Format for Medical Record

(See Regulation 1.3.1)

Name of the patient

:

Age

:

Sex

:

Address

:

Occupation

:

Date of 1st visit

:

Clinical Note (Summary) of the case

:

Provisional diagnosis

:

Investigations advised with reports

:

Diagnosis after investigation

:

Advice

:

Follow up

:

Observations

:

Date

:

Signature in full.....

Name of Treating Physician

Appendix-4

List of certificates, reports, notifications, etc. issued by doctors for the purposes of various acts/administrative requirements

(a)

Under the Acts relating to birth, death or disposal of the dead.

(b)

Under the Acts relating to Lunacy and Mental Deficiency and under the Mental illness Act and the Rules made thereunder.

(c)

Under the vaccination Acts and the Regulations made thereunder.

(d)

Under the Factory Acts and the Regulations made thereunder.

(e)

Under the Education Acts.

(f)

Under the Public Health Acts and the orders made thereunder.

(g)

Under Workmen's Compensation Act and Persons with Disability Act.

(h)

Under the Acts and orders relating to the notification of infectious diseases.

(i)

Under the Employee's State Insurance Act.

(j)

In connection with sick benefit insurance and friendly societies.

(k)

Under the Merchant Shipping Act.

(l)

For procuring/issuing of passports.

(m)

For excusing attendance in Courts of Justice, in public services, in public officers or in ordinary employment.

(n)

In connection with Civil and Military matters.

(o)

In connection with matters under the control of Department of Pensions.

(p)

In connection with quarantine rules.

(q)

For procuring driving licence.

Important Notice

Every registered Medical Practitioner should be careful to send to the Registrar immediate notice of any change in his address and also answer all enquiries that may be sent to him by the Registrar in regard thereto in order that his correct address may be duly inserted in the Register of Registered Practitioners.

Related AI tags, queries and research notes

Translation